2.15 IV Therapy and Central Venous Lines*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "The area around the cannula is sore." **Objective** • Site red and swollen • Surrounding skin cool and pale • Drip slowing	**ACTUAL** Impaired tissue integrity related to infiltration or phlebitis as evidenced by swelling, erythema and pain at the site.	• Site resolved, without redness or swelling, within 48 hours.	• Stop the infusion and remove the cannula. • Elevate the limb and apply a compress as policy directs. • Re-site in the other limb if access is still needed. • Grade infiltration or phlebitis on a validated scale and document it.	• Goal reached as the site settles.
Subjective • "What should I do if the line pulls?" **Objective** • PICC newly inserted • Cannot describe how to look after it	**ACTUAL** Inadequate health knowledge related to care of a vascular access device as evidenced by questions about line care.	• Before discharge, names the warning signs and how to protect the line.	• Teach her to keep the dressing dry and intact and to cover it when showering. • Teach signs of infection (redness, discharge, fever) and of occlusion. • Warn against pulling or kinking the line and against unauthorized access. • Give written instructions and a contact number.	• Met when line care is explained correctly.
Subjective *No specific complaint* **Objective** • Central venous catheter • Long-term IV therapy	**RISK** Risk for infection as evidenced by risk factors: invasive vascular device.	• No catheter-related bloodstream infection.	• Use hand hygiene and aseptic non-touch technique at every access. • Scrub the hub before each use. • Change the dressing per policy, or sooner if soiled. • Inspect the site daily and question whether the line is still needed. • Monitor temperature.	• Achieved if no infection develops.

* **References for this care plan:** [35], [105], [107], [110]. Full entries are listed in the References section of the CPMN book.